

1. A 72-year-old man with a 10-day history of fever, nausea and vomiting, and progressive confusion remains hospitalized because his symptoms have not resolved despite treatment with broad-spectrum antibiotics. He has a 6-year history of severe emphysema treated with courses of corticosteroids during the past year. Medications include intravenous ceftriaxone and ampicillin, albuterol inhalers, theophylline, and prednisone. He is confused and stuporous. His temperature is 38.9°C (102°F), pulse is 84/min, and blood pressure is 142/86 mm Hg. Physical examination shows meningismus. An MRI of the brain shows diffuse meningeal enhancement with a basilar predominance. A lumbar puncture is done; cerebrospinal fluid analysis shows a leukocyte count of 322/mm³ (86% lymphocytes), a glucose concentration of 17 mg/dL, and a protein concentration of 212 mg/dL. Which of the following is the most likely diagnosis?
- ☐ A) Herpes simplex encephalitis
 - ☐ B) Lyme disease
 - ☐ C) Neurosarcoidosis
 - ☐ D) Neurosyphilis
 - ☐ E) Tuberculous meningitis



Next

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Lab Values



Calculator



Review



Help



Pause

2. An 8-month-old girl is brought to the physician by her parents because of increased irritability over the past month. During this period, she has had intermittent low-grade fever and episodes of stiffening of her body. Her father reports that she previously interacted with them and smiled when prompted but has not made eye contact for the past 3 days. She has a history of hypotonia. She cannot yet roll over or sit up. Her parents are third cousins. She is awake and irritable and has poor visual regard. Examination of the skin shows no neurocutaneous lesions. The optic nerves are pale. Neurologic examination shows hyperextension of the head and back, ankle spasticity, and fisting. Deep tendon reflexes are brisk, and Babinski sign is present. An MRI of the brain is most likely to show which of the following?

- ☐ A) Cerebellar hypoplasia
- ☐ B) Degeneration of the white matter
- ☐ C) Holoprosencephaly
- ☐ D) Hydrocephalus
- ☐ E) Periventricular calcifications
- ☐ F) Schizencephaly
- ☐ G) No abnormalities



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

3. A 57-year-old woman is brought to the emergency department 20 minutes after a simple partial seizure. Examination shows mild weakness of the right lower extremity. A CT scan of the head shows a 1.5-cm, uniformly and intensely enhancing parasagittal mass in the left frontal region. Which of the following is the most likely diagnosis?

- ☐ A) Astrocytoma
- ☐ B) Ependymoma
- ☐ C) Glioblastoma multiforme
- ☐ D) Meningioma
- ☐ E) Schwannoma



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Lab Values



Calculator



Review



Help



Pause

4. A 67-year-old semiretired corporate executive comes to the physician with his wife because she is concerned about his increasing disorganization at home over the past year. She also notes that he is unable to remember the names of recent acquaintances. He has insisted on working part-time since stepping down from a CEO position. His secretary has told his wife that he has been forgetting scheduled appointments and becomes angry and tearful when he is reminded. He drinks one to two cocktails each evening. Physical examination shows no abnormalities. On mental status examination, he is mildly agitated and has word-finding difficulty and short-term memory deficits. He is oriented to person, place, and time. Which of the following is the most likely diagnosis?

- | | |
|---|--|
| ☐ A) Acute stress disorder | ☐ J) Hypercalcemia |
| ☐ B) Alcohol-induced amnestic disorder | ☐ K) Major depressive disorder |
| ☐ C) Dementia, Alzheimer type | ☐ L) Multi-infarct (vascular) dementia |
| ☐ D) Dissociative amnesia | ☐ M) Niacin deficiency |
| ☐ E) General paresis | ☐ N) Normal-pressure hydrocephalus |
| ☐ F) Head trauma | ☐ O) Parkinson disease |
| ☐ G) Hepatolenticular degeneration (Wilson disease) | ☐ P) Pick disease |
| ☐ H) HIV encephalitis | ☐ Q) Normal aging |
| ☐ I) Huntington disease | |



5. A 17-year-old boy is brought to the emergency department by ambulance 2 hours after he dove into a shallow lake and struck the back of his head on the bottom. He immediately had the onset of pain in his neck and a sensation of an electric shock radiating from his neck to his left shoulder. He did not have loss of consciousness. He was able to stand, and two friends helped him out of the water into a supine position on the dock. At the scene, his spine was immobilized with a cervical collar, and his head was taped to a backboard. On arrival, he is alert. Vital signs are within normal limits. Muscle strength is 3/5 on extension of the left wrist. Deep tendon reflexes are normal. Babinski sign is absent. Sensorimotor examination of the right upper extremity and lower extremities shows no abnormalities. Sensation to light touch is decreased over the radial aspect of the left forearm and hand and over the palmar and dorsal aspects of the left thumb. Rectal examination shows normal sphincter tone and sensation. A lateral x-ray of the cervical spine is shown. This patient most likely has which of the following cervical spine injuries?

- ☐ A) Anterior spinal artery thrombosis
- ☐ B) Bilateral dislocation of facets with impingement of the left C5 nerve root
- ☐ C) Bilateral dislocation of facets with impingement of the left C6 nerve root
- ☐ D) Brown-Séquard syndrome
- ☐ E) Unilateral dislocation of a facet with impingement of the left C5 nerve



6. A 62-year-old man is brought to the emergency department 2 hours after the sudden onset of headache and left-sided weakness that have become increasingly severe. He has hypertension treated with lisinopril and chronic neck pain treated with 325-mg aspirin. On arrival, he appears mildly lethargic. His pulse is 60/min, respirations are 16/min, and blood pressure is 180/100 mm Hg. Examination shows moderate weakness of the lower left side of the face. Muscle tone is decreased in the left upper and lower extremities. Muscle strength is 4/5 on the left and 5/5 on the right. Deep tendon reflexes are 3+ on the left. Babinski sign is present on the left. A CT scan of the head without contrast is shown. Which of the following is the most likely diagnosis?

- ☐ A) Cerebral amyloid angiopathy
- ☐ B) Hypertensive intracerebral hematoma
- ☐ C) Ischemic cerebral infarction
- ☐ D) Ruptured arteriovenous malformation
- ☐ E) Ruptured berry aneurysm



7. Over the past 6 months, an otherwise healthy 19-year-old cheerleader has had increasingly severe neurologic symptoms. Initially, she had difficulty jumping; 2 weeks later she had a tingling feeling in the toes of her right foot that quickly spread to the left foot. Over the past 2 weeks, she has stumbled and fallen several times. Examination of the upper extremities shows normal findings. Deep tendon reflexes are absent in the lower extremities. She is unable to heel walk and has mild difficulty walking on her toes and rising from a low squatting position. Position sensation is normal, but pinprick feels less sharp distally and occasional light touch stimuli are missed over the toes. Peroneal motor conduction velocity is moderately slowed (22 m/sec; N=42–57). Which of the following is the most likely diagnosis?

- | | |
|--|---|
| ☐ A) Chronic inflammatory demyelinating polyradiculoneuropathy | ☐ G) Spinal cord tumor |
| ☐ B) Diabetic neuropathy | ☐ H) Spinal epidural abscess |
| ☐ C) Epidural spinal cord metastasis | ☐ I) Spondylotic myelopathy |
| ☐ D) Guillain-Barré syndrome | ☐ J) Syringomyelia |
| ☐ E) Multiple sclerosis | ☐ K) Vasculitic neuropathy |
| ☐ F) Spinal cord infarction | ☐ L) Vitamin B ₁₂ (cobalamin) deficiency |



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

8. A 62-year-old man is brought to the emergency department because of progressive weakness of his legs during the past week. He has chronic kidney disease and receives dialysis three times weekly via a percutaneous tunneled catheter. Last week, he had an episode of sweating and chills. His temperature is 38.9°C (102°F), pulse is 100/min and regular, and blood pressure is 150/92 mm Hg. Examination shows severe tenderness to percussion over the lumbar spine. The catheter site appears normal. There is trace edema of the lower extremities. Muscle strength is 3/5 on flexion and extension of the knees and ankles. Deep tendon reflexes are increased at the patellae. Laboratory studies show:

Hemoglobin	11.1 g/dL
Leukocyte count	19,200/mm ³
Segmented neutrophils	90%
Bands	5%
Eosinophils	1%
Lymphocytes	4%
Platelet count	290,000/mm ³

An MRI of the spine shows an epidural abscess at L4. Blood cultures grow *Staphylococcus aureus*. In addition to antibiotic therapy and drainage of the abscess, which of the following is the most appropriate next step in management?

- ☐ A) Removal of the catheter
- ☐ B) Electromyography
- ☐ C) Technetium 99m bone scan
- ☐ D) Bone biopsy of L4
- ☐ E) Lumbar puncture



9. A 67-year-old man is brought to the emergency department by his wife 30 minutes after a 15-minute episode of inability to speak. He has a 15-year history of poorly controlled hypertension. Three weeks ago, he had a transmural myocardial infarction and underwent placement of a stent in the left anterior descending coronary artery. Current medications include clopidogrel, benazepril, and 81-mg aspirin. He has smoked two packs of cigarettes daily for 30 years. On arrival, he is alert and oriented to person, place, and time. His pulse is 60/min, respirations are 15/min, and blood pressure is 160/100 mm Hg. A grade 1/6 systolic murmur is heard best at the second left intercostal space. Neurologic examination shows no focal findings. Carotid duplex ultrasonography shows 40% stenosis of the left carotid artery. Which of the following is the most likely diagnosis?

- ☐ A) Embolus from a ventricular aneurysm
- ☐ B) Infarct of the lacunar artery
- ☐ C) Intraparenchymal cerebral hemorrhage
- ☐ D) Spasm of a cerebral artery
- ☐ E) Subarachnoid hemorrhage



10. One month after starting isoniazid and rifampin therapy for pulmonary tuberculosis, a 62-year-old woman develops weakness, fatigue, and paresthesias. Examination shows pallor and diminished light touch sensation in the extremities. Laboratory studies show:

Hemoglobin	9 g/dL
Mean corpuscular volume	78 μm^3
Leukocyte count	8000/mm ³
Platelet count	210,000/mm ³

These symptoms are most likely to respond to treatment with which of the following?

- ☐ A) Folic acid
- ☐ B) Iron
- ☐ C) Vitamin B₁ (thiamine)
- ☐ D) Vitamin B₆
- ☐ E) Vitamin B₁₂ (cyanocobalamin)



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

11. A 22-year-old man who is a collegiate volleyball player comes to the physician because of a 2-month history of progressive pain and weakness in his right shoulder. There is no history of trauma. He is 196 cm (6 ft 5 in) tall and weighs 95 kg (210 lb); BMI is 25 kg/m². There is significant loss of muscle mass in the dorsal scapula region, especially in the area of the scapular spine. Passive abduction and external rotation of the right shoulder and protraction of the scapula reproduce the pain. There is weakness with resisted external rotation and initiation of abduction. Active range of motion shows full flexion and abduction. Sensory examination shows no cutaneous deficits along the right side of the neck or in the right upper extremity. Which of the following is the most likely cause of this patient's symptoms?

- ☐ A) Adhesive capsulitis
- ☐ B) Axillary nerve damage
- ☐ C) Brachial plexus palsy
- ☐ D) Herniated cervical disc
- ☐ E) Irritation of the suprascapular nerve



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

12. A 32-year-old woman comes to the physician with her husband because of a 6-month history of fluctuating symptoms including fatigue, loss of balance, blurred vision, and the sensation of pins and needles over her arms and legs. Her husband says that she has also had mood swings with episodes of uncontrollable crying over the past 2 months. Her temperature is 37.3°C (99.2°F), pulse is 80/min, respirations are 16/min, and blood pressure is 128/86 mm Hg. Neurologic examination shows reduced medial gaze of the left eye and numbness to pinprick over the left distal lower extremities. Romberg sign is present. Babinski sign is present on the left. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) EEG
- ☐ B) Measurement of serum ammonia concentration
- ☐ C) Measurement of serum calcium concentration
- ☐ D) Measurement of serum copper concentration
- ☐ E) Measurement of serum vitamin B₁₂ (cobalamin) concentration
- ☐ F) Measurement of urine porphobilinogen concentration
- ☐ G) MRI of the brain
- ☐ H) Serum antinuclear antibody assay
- ☐ I) VDRL test in cerebrospinal fluid



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

13. A physician makes a house call for an 87-year-old woman who is unable to come to the office because of difficulty with transportation. Her family reports that she has fallen frequently during the past 6 weeks. The patient says that she has had some difficulty feeling where her feet are but that she is otherwise well. She has not had loss of consciousness. She has no history of serious illness and takes no medications. Vital signs are within normal limits. Muscle strength is 4/5 on flexion of the hips and extension of the knees and ankles bilaterally. Proprioception and sensation to vibration are decreased in the lower extremities. Sensation to light touch is intact. She has a slapping gait. Mental status examination shows no abnormalities. The physician counsels the patient about making enhancements to her home to prevent falls. Which of the following is the most appropriate next step in diagnosis?

- ☐ A) Determination of erythrocyte sedimentation rate
- ☐ B) Measurement of serum vitamin B₁₂ (cobalamin) concentration
- ☐ C) X-rays of the lumbosacral spine
- ☐ D) EEG
- ☐ E) Electromyography



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

14. A 27-year-old nulligravid woman has had café au lait spots and multiple neurofibromas since birth. A diagnosis of neurofibromatosis is made. She is concerned that she might pass this condition on to her children. Her parents and three siblings do not have this disorder. The risk for neurofibromatosis in her children is closest to which of the following?
- ☐ A) 50% for each child, since neurofibromatosis is autosomal dominant
 - ☐ B) 25% for each child, since neurofibromatosis is autosomal recessive
 - ☐ C) 10% for each child, since neurofibromatosis is controlled by several genes
 - ☐ D) No risk, since neurofibromatosis is not a genetic disorder
 - ☐ E) No risk, since the patient represents a new mutation



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

15. A 57-year-old man comes to the physician 2 hours after the sudden onset of visual loss in his right eye. He has no history of similar episodes. He has a 12-year history of type 2 diabetes mellitus treated with metformin and hypertension treated with lisinopril. He sustained a myocardial infarction 2 years ago. His pulse is 90/min, and blood pressure is 170/96 mm Hg. Findings on right funduscopic examination are shown. Which of the following is the most likely diagnosis?

- | | |
|---|---|
| ☐ A) Acute angle-closure glaucoma | ☐ F) Endophthalmitis |
| ☐ B) Bacterial conjunctivitis | ☐ G) Idiopathic intracranial hypertension |
| ☐ C) Cataract | ☐ H) Orbital melanoma |
| ☐ D) Central retinal artery occlusion | ☐ I) Sarcoidosis |
| ☐ E) Diabetic retinopathy | ☐ J) Temporal arteritis |



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

16. A 52-year-old woman comes to the physician because of facial numbness for 2 weeks. Her symptoms began as a "funny feeling" over a patch of skin on the left side of her chin and have gradually progressed to a "novocaine" sensation. She has had no facial pain or numbness of the gums or oral mucosa. She has a 3-year history of breast cancer metastatic to the lungs treated with tamoxifen. Examination shows supraclavicular adenopathy. Sensation to pinprick, light touch, and temperature is absent over the left side of the chin. Neurologic examination shows no other abnormalities. Which of the following is the most likely location of the abnormality?

- ☐ A) Brachial plexus
- ☐ B) Cavernous sinus
- ☐ C) Cerebellopontine angle
- ☐ D) Mandible
- ☐ E) Parietal lobe



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

17. A 32-year-old woman with AIDS comes to the physician because of a 2-week history of painful burning sensations in her hands and feet. Examination shows no abnormalities. Neurologic examination shows no focal findings. Which of the following is the most appropriate initial pharmacotherapy?

- ☐ A) Oral fluoxetine
- ☐ B) Oral lorazepam
- ☐ C) Oral nortriptyline
- ☐ D) Oral prednisone
- ☐ E) Topical lidocaine to the affected areas



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

18. A 37-year-old man comes to the physician because of a 1-week history of moderate low back pain. He thinks he may have hurt his back while doing yard work. His pain moderately improves with rest; he has been spending a great amount of time in bed. He has not had other symptoms, weight loss, or a recent infection. He has no history of serious illness. His only medication is ibuprofen for the pain. He does not use intravenous drugs or anabolic steroids. He is 178 cm (5 ft 10 in) tall and weighs 102 kg (225 lb); BMI is 32 kg/m². Examination shows mild paraspinal muscle tenderness. Neurologic examination shows no focal findings. Which of the following is the most appropriate next step in management?

- ☐ A) Bed rest for 10 days
- ☐ B) X-ray of the lumbar spine
- ☐ C) MRI of the lumbar spine
- ☐ D) Continuation of ibuprofen therapy
- ☐ E) Oxycodone therapy as needed for pain



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

19. A 37-year-old woman comes to the physician for a follow-up examination 3 weeks after operative repair of a circle of Willis berry aneurysm. Her husband says that her behavior has changed since the operation. She is unmotivated and just sits around all day like a "bump on a log." She has not had depressed mood. Examination shows psychomotor retardation. She is oriented to person, place, and time. She is cooperative and her speech is fluent and grammatical, but there is little spontaneous speech during conversation. Which of the following cerebral cortical regions is most likely affected?

- ☐ A) Frontal
- ☐ B) Insular
- ☐ C) Occipital
- ☐ D) Parietal
- ☐ E) Temporal

20. A 10-day-old newborn is brought to the emergency department 1 hour after a seizure that involved clonic jerking of the left arm. He had been well until 2 days ago when he began sleeping somewhat more but continued to feed well. Pregnancy, labor, and delivery were uncomplicated. Vital signs are within normal limits. He is difficult to awaken but moves all extremities when stimulated. He is hypotonic but hyperreflexic. Babinski sign is present. During examination, he has a clonic seizure of the right lower extremity. A CT scan of the head shows no abnormalities. Cerebrospinal fluid analysis shows:

Glucose	55 mg/dL
Protein	110 mg/dL
Leukocyte count	85/mm ³
Erythrocyte count	860/mm ³

A Gram stain is negative. Which of the following is the most likely diagnosis?

- ☐ A) Congenital infection with cytomegalovirus
- ☐ B) Congenital infection with syphilis
- ☐ C) Congenital infection with toxoplasmosis
- ☐ D) Neonatal *Escherichia coli* meningitis
- ☐ E) Neonatal group B streptococcus meningitis
- ☐ F) Neonatal herpes simplex encephalitis



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

21. A 47-year-old woman comes to the physician because of increasingly frequent headaches during the past 3 months. The headaches are throbbing, occur only on the right side, and are associated with nausea, vomiting, and sensitivity to light and sound. She has had one or two headaches yearly since the age of 17 years but now has two or three weekly. She is a schoolteacher and has had no changes in work, lifestyle, or environment. Four months ago, conjugated estrogen therapy was begun for treatment of menopausal symptoms. Her temperature is 37°C (98.6°F), pulse is 76/min, respirations are 14/min, and blood pressure is 135/80 mm Hg. Neurologic examination shows no abnormalities. Which of the following is the most appropriate next step in management?

- ☐ A) CT scan of the head
- ☐ B) Butalbital therapy combined with acetaminophen and caffeine
- ☐ C) Discontinuation of conjugated estrogen therapy
- ☐ D) EEG
- ☐ E) Lumbar puncture



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

22. A 67-year-old man is brought to the physician by his wife because of a 1-month history of deafness, ringing in both ears, dizziness, and difficulty with balance. He also has had an occasional sensation that the room is spinning. He was discharged from the hospital 21 days ago after prolonged treatment for pneumonia and sepsis with intravenous ceftriaxone and gentamicin. He has chronic obstructive pulmonary disease, type 2 diabetes mellitus, and congestive heart failure. He has a history of tuberculosis 10 years ago. Medications include digoxin, metformin, and losartan. His pulse is 84/min, respirations are 20/min, and blood pressure is 140/85 mm Hg. Examination shows nystagmus on lateral gaze bilaterally. There is severe loss of hearing bilaterally. Air conduction is greater than bone conduction. Results of Weber test are normal. Muscle strength, deep tendon reflexes, and sensory findings are normal. His gait is broad-based, and he is unable to walk in tandem. Which of the following is the most likely diagnosis?

- ☐ A) Aminoglycoside-induced ototoxicity
- ☐ B) Cerebellopontine angle tumor
- ☐ C) Meniere disease
- ☐ D) Pontine infarction
- ☐ E) Vestibular neuronitis



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

23. A 72-year-old woman comes to the physician because of a 2-day history of pain in the left thigh and flank and weakness of the left leg. She has a 30-year history of type 2 diabetes mellitus currently controlled with glyburide. Examination shows weakness of the left iliopsoas and quadriceps muscles. Deep tendon reflex of the left quadriceps muscle is decreased. Sensation to pinprick is decreased over the left lateral thigh and medial calf. Which of the following is the most likely location of the lesion?

- | | |
|--|---|
| ☐ A) Axillary nerve | ☐ H) C7 Radiculopathy |
| ☐ B) Femoral nerve | ☐ I) L5 Radiculopathy |
| ☐ C) Lateral femoral cutaneous nerve | ☐ J) S1 Radiculopathy |
| ☐ D) Median nerve | ☐ K) Sciatic nerve |
| ☐ E) Peroneal nerve | ☐ L) Tibial nerve |
| ☐ F) Radial nerve | ☐ M) Ulnar nerve |
| ☐ G) C6 Radiculopathy | |



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

24. A 72-year-old man comes to the physician because of a 2-month history of a progressive tremor of his hands. He and his family have noticed that his movements have become more shaky during this period and that he has been walking more slowly during the past year. He has hypertension. Current medications include hydrochlorothiazide and aspirin. There is no family history of serious illness. He does not smoke or drink alcohol. His pulse is 56/min, and blood pressure is 140/90 mm Hg. Examination shows a fine tremor of the hands at rest; the tremor is exacerbated with movement. There is cogwheel rigidity of the upper extremities. His gait is slow and shuffling. Mental status examination shows no abnormalities. Which of the following is the most appropriate initial pharmacotherapy?

- ☐ A) Atenolol
- ☐ B) Carbidopa-levodopa
- ☐ C) Donepezil
- ☐ D) Nifedipine
- ☐ E) Primidone



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25. A 52-year-old man comes to the physician because of a 2-hour history of a diffuse, bilateral, dull headache. He has hypertension and has been poorly compliant with his medication. He is alert but confused. His blood pressure is 240/150 mm Hg. Fundusoscopic examination shows hemorrhages and exudates. Which of the following is the most likely diagnosis?

- ☐ A) Cerebral hemorrhage
- ☐ B) Cerebral infarction
- ☐ C) Hypertensive encephalopathy
- ☐ D) Idiopathic intracranial hypertension
- ☐ E) Migraine
- ☐ F) Subarachnoid hemorrhage
- ☐ G) Subdural hematoma



26. A 67-year-old woman comes to the physician because of excessive sleepiness and fatigue. She has required more sleep for several years, but the problem has become increasingly worse during the past 7 months. She sometimes falls asleep in the middle of conversations and at dinner. She recently collapsed onto the floor of her apartment; she was aware of her surroundings but unable to move for 5 minutes. She has no history of seizures or other neurologic disorders. Her only medication is celecoxib for treatment of osteoarthritis. She appears well nourished. Physical examination shows limited range of motion in the lower extremities. There is swelling and crepitus of the knees. She has moderate pain on standing. An ECG, EEG, and 72-hour ambulatory ECG monitoring show no abnormalities. Polysomnography is pending. Which of the following is the most appropriate pharmacotherapy?

- ☐ A) Methylphenidate
- ☐ B) Mirtazapine
- ☐ C) Nortriptyline
- ☐ D) Trazodone
- ☐ E) Zaleplon



Previous

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Next

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Lab Values



Calculator



Review



Help



Pause

27. A 72-year-old right-handed woman is brought to the emergency department 30 minutes after the onset of double vision and right-sided weakness that began while she was eating breakfast. She has type 2 diabetes mellitus and hypertension. Medications include glyburide and atenolol. Her temperature is 37°C (98.6°F), pulse is 80/min and regular, and blood pressure is 170/105 mm Hg. She is alert and oriented. Her speech is fluent but mildly slurred. She can follow complex commands. Cranial nerve examination shows left ptosis. Adduction and elevation of the left eye are impaired. Ocular movements are full on the right. The right pupil is 3 mm, and the left pupil is 5 mm. The right pupil reacts normally, but the left pupil does not constrict either to direct or consensual light stimulation. There is drooping of the right side of the mouth. Muscle strength is normal in the left upper and lower extremities and is 4/5 in the right upper and lower extremities. Deep tendon reflexes are 3+ on the right and 2+ on the left. Babinski sign is present on the right and absent on the left. There is no ataxia. Sensation is intact. Which of the following is the most likely location of this patient's lesion?

- ☐ A) Left cerebral cortex
- ☐ B) Left internal capsule
- ☐ C) Left medulla
- ☐ D) Left midbrain
- ☐ E) Left pons



Previous



Next

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Lab Values



Calculator



Review



Help



Pause

28. Over the past 3 months, a 40-year-old woman has had multiple 1- to 2-minute episodes of numbness and tingling that begin in the right hand and then spread to the right arm and the face. Neurologic examination shows normal findings. The most likely cause is a lesion at which of the following sites?

- ☐ A) Frontal lobe
- ☐ B) Medulla
- ☐ C) Midbrain
- ☐ D) Occipital lobe
- ☐ E) Parietal lobe
- ☐ F) Pons
- ☐ G) Retina
- ☐ H) Temporal lobe



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

29. A 72-year-old man comes to the physician because of a 3-month history of increasingly depressed mood, fatigue, difficulty concentrating, and a decreased appetite resulting in a 2.3-kg (5-lb) weight loss. He was diagnosed with Parkinson disease 2 years ago. He also has hypertension and benign prostatic hyperplasia. His medications are amlodipine, carbidopa-levodopa, and tamsulosin. He has smoked one and one-half packs of cigarettes daily for 47 years. He states that he would like to stop smoking. Vital signs are within normal limits. Physical examination shows mild cogwheel rigidity, resting tremor of the hands, and gait instability. On mental status examination, he has a depressed mood and forlorn affect. Results of laboratory studies, including thyroid function tests, are within the reference ranges. Addition of which of the following is the most appropriate next step in pharmacotherapy?

- ☐ A) Bupropion
- ☐ B) Citalopram
- ☐ C) Duloxetine
- ☐ D) Lorazepam
- ☐ E) Mirtazapine



30. A 72-year-old woman is brought to the physician by her husband because of visual hallucinations during the past 2 weeks. She has a 10-year history of hypertension and a 5-year history of Parkinson disease. Current medications include hydrochlorothiazide, aspirin, metoprolol, and carbidopa-levodopa. Three weeks ago, her carbidopa-levodopa dose was increased because of decreased mobility, and metoprolol was added to her medication regimen because of inadequate blood pressure control. She smoked one pack of cigarettes daily for 40 years but quit 10 years ago. She drinks one glass of wine with dinner every night. Her temperature is 36.8°C (98.2°F), pulse is 58/min, respirations are 18/min, and blood pressure is 148/80 mm Hg. Pulse oximetry on room air shows an oxygen saturation of 92%. Neurologic examination shows a resting tremor of the hands and cogwheel rigidity. Deep tendon reflexes are absent at the ankles. Her gait is shuffling. Mental status examination shows a constricted affect. She is oriented to person but not to place or time. She can recall one of three objects after 5 minutes. She says she sees ghost-like figures entering the room. Results of laboratory studies are within the reference range. A CT scan of the head shows mild atrophy. Which of the following is the most likely cause of this patient's visual hallucinations?

- ☐ A) Adverse effect of carbidopa-levodopa
- ☐ B) Major depressive disorder
- ☐ C) Multi-infarct (vascular) dementia
- ☐ D) Normal-pressure hydrocephalus
- ☐ E) Progression of Parkinson disease



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

31. A 77-year-old man comes to the physician because of increasingly severe low back pain for 3 months. He has had poor control of bowel and bladder function for 2 weeks. He fell yesterday on his way to the bathroom. He has smoked two packs of cigarettes daily for 60 years. Examination shows marked weakness of the lower extremities, normal upper extremity strength, bilateral brisk deep tendon reflexes in the lower extremities, a sensory level at T9, and a flaccid anal sphincter. There is tenderness over the lower thoracic spine. Babinski sign is present. Which of the following is the most likely diagnosis?

- ☐ A) Carcinomatous meningitis
- ☐ B) Cervical spondylosis
- ☐ C) Epidural metastatic carcinoma
- ☐ D) Lumbar spinal stenosis
- ☐ E) Myasthenia gravis
- ☐ F) Syringomyelia
- ☐ G) Tabes dorsalis
- ☐ H) Transverse myelitis



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

32. A 47-year-old man is brought to the emergency department by his friend 30 minutes after being found unconscious in his bed. The patient lives alone in a trailer park, and his friend says he was well the night before. The patient has no history of serious illness and takes no medications. He does not smoke cigarettes or use illicit drugs. He appears well nourished. His temperature is 37°C (98.6°F), pulse is 100/min, respirations are 20/min, and blood pressure is 110/60 mm Hg. The nail beds and oral mucosa are pink. Neurologic examination shows no meningismus. The pupils are 4 mm and sluggishly reactive. The doll's eye (oculocephalic) reflex is intact. There is no verbalization or eye opening in response to painful stimuli. He withdraws all extremities symmetrically to painful stimuli. Deep tendon reflexes are 3+ throughout. Babinski sign is absent bilaterally. Results of laboratory studies are within the reference ranges. Arterial blood gas analysis on room air shows a P_{CO_2} of 30 mm Hg and P_{O_2} of 80 mm Hg. In addition to supportive care, which of the following is the most appropriate therapy for this patient's condition?

- ☐ A) Hyperbaric oxygen
- ☐ B) Methylene blue
- ☐ C) Naloxone
- ☐ D) Physostigmine
- ☐ E) Vitamin B₁ (thiamine)



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

33. A 5-year-old girl has the insidious onset of a clumsy gait over the past 3 months. Examination shows pes cavus of the left foot. There is generalized diminished girth of the left leg, and a decreased tendon reflex of the left ankle. The tendon reflexes at the knees are brisk. There is a sacral tuft of hair. Which of the following is the most likely diagnosis?

- ☐ A) Ataxia-telangiectasia
- ☐ B) Friedreich ataxia
- ☐ C) Juvenile rheumatoid arthritis
- ☐ D) Spastic diplegia
- ☐ E) Tethered spinal cord



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

34. A 42-year-old woman with AIDS comes to the physician because of a 2-week history of progressive right-sided weakness, slurred speech, and headache. She is right-handed. Funduscopic examination shows papilledema. Neurologic examination shows aphasia, right homonymous hemianopia, and right-sided spastic weakness. A CT scan of the head with contrast shows multiple small ring-enhancing lesions with surrounding edema and mass effect in the left frontal, right occipital, and left parietal lobes. Which of the following is the most likely diagnosis?

- ☐ A) Cryptococcosis
- ☐ B) Cysticercosis
- ☐ C) Primary central nervous system lymphoma
- ☐ D) Progressive multifocal leukoencephalopathy
- ☐ E) Toxoplasmosis



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

35. A 52-year-old man is brought to the emergency department because of a 4-day history of progressive numbness and weakness of his legs. Six years ago, he had non-Hodgkin lymphoma treated successfully with chemotherapy and radiation therapy. He is currently taking no medications. Cranial nerve examination shows no abnormalities. Muscle strength is intact in the upper extremities but 3/5 in the proximal lower extremities and 4/5 in the distal lower extremities. Deep tendon reflexes are 2+ in the upper extremities and 3+ in the lower extremities. Babinski sign is present bilaterally. Sensation to pinprick is decreased below the level of the nipples, and sensation to vibration is decreased over the feet and ankles. Which of the following is the most likely diagnosis?

- ☐ A) Chemotherapy-induced polyneuropathy
- ☐ B) Guillain-Barré syndrome
- ☐ C) Metastatic spinal cord compression
- ☐ D) Myopathy
- ☐ E) Paraneoplastic neuropathy



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

36. A previously healthy 62-year-old man has had progressive difficulty with speech and swallowing over the past year. Symptoms were subtle at first, and only he and his family noticed a problem. Recently, two ham radio colleagues have commented that he sounds different on the air. He has also noticed that he has difficulty manipulating objects in the left hand, walking smoothly, and going up and down stairs. He has not had any difficulty with his thought processes. Examination shows slow deliberate speech with mild dysarthria. Cranial nerves are otherwise normal. There is wasting of intrinsic hand muscles and distal weakness in the left hand. Gait is slightly broad-based, but Romberg sign is absent. Deep tendon reflexes are 3+ in the left upper extremity, 2+ in the right upper extremity, and 3+ in the lower extremities. Jaw jerk is hyperactive. His serum potassium concentration is 3.2 mEq/L. An x-ray of the chest shows normal findings. Which of the following is the most likely diagnosis?

- | | |
|--|---|
| ☐ A) Alcoholic rhabdomyolysis | ☐ J) Hypokalemic periodic paralysis |
| ☐ B) Amyotrophic lateral sclerosis | ☐ K) Hypothyroid myopathy |
| ☐ C) Becker muscular dystrophy | ☐ L) Lumbar spinal stenosis |
| ☐ D) Botulism | ☐ M) Multiple sclerosis |
| ☐ E) Carcinomatous meningitis | ☐ N) Myasthenia gravis |
| ☐ F) Cervical spondylosis | ☐ O) Polymyositis |
| ☐ G) Epidural metastatic carcinoma | ☐ P) Sensorimotor peripheral neuropathy |
| ☐ H) Epstein-Barr virus infection | ☐ Q) Spinal cord compression |
| ☐ I) Guillain-Barré syndrome | |

37. A 27-year-old man is admitted to the hospital because of a 4-week history of difficulty with speech and weakness of his right leg. He has a 5-year history of HIV infection treated with antiretroviral therapy. On admission, his temperature is 37°C (98.6°F). Examination shows thrush. There is mild nonfluent aphasia. Muscle strength is 4/5 in the right lower extremity. Deep tendon reflexes are 3+ in the right lower extremity and 2+ elsewhere. Babinski sign is present on the right. An MRI of the brain shows six ring-enhancing lesions in the cerebral hemispheres bilaterally; the largest lesion is in the left frontal lobe. Some of the lesions have surrounding edema. Pyrimethamine and sulfadiazine therapy is started. Two weeks later, there is no improvement in his symptoms. A second MRI of the brain shows no decrease in the size of the lesions, and some have increased in size. Which of the following is the most likely diagnosis?

- ☐ A) Candidal abscesses
- ☐ B) Central nervous system lymphoma
- ☐ C) Cryptococcal meningitis
- ☐ D) Listerial meningoencephalitis
- ☐ E) Nocardial abscesses
- ☐ F) Toxoplasma encephalitis

38. A 67-year-old woman comes for a routine health maintenance examination. She is concerned about developing dementia because she lives alone; she does not want to be a burden to her children. Her father was just diagnosed with dementia, Alzheimer type. She takes no medications other than hydrochlorothiazide for hypertension. Her blood pressure is 124/84 mm Hg. Examination shows no abnormalities. Which of the following measures is most likely to decrease her risk for multi-infarct (vascular) dementia?

- ☐ A) Continued control of blood pressure
- ☐ B) Regular aerobic exercise
- ☐ C) Daily aspirin prophylaxis
- ☐ D) Daily fish oil supplementation
- ☐ E) Daily vitamin D and calcium supplementation
- ☐ F) Daily vitamin E supplementation



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

39. A previously healthy 45-year-old woman is brought to the emergency department 1 hour after she lost consciousness while shopping in a department store; on regaining consciousness, she had a severe headache and photophobia. Her temperature is 37°C (98.6°F), pulse is 110/min, respirations are 14/min, and blood pressure is 110/70 mm Hg. Examination shows slight neck stiffness with flexion. There is pain with lateral deviation of the eyes. Neurologic examination shows no other abnormalities. A CT scan of the head shows no lesions or evidence of bleeding. Which of the following is the most appropriate next step in management?

- ☐ A) Admission for observation
- ☐ B) Discharging the patient with analgesic therapy
- ☐ C) Carotid ultrasonography
- ☐ D) MRI of the brain
- ☐ E) Lumbar puncture



40. A 72-year-old woman is brought to the emergency department 6 hours after the onset of double vision, loss of balance, and dizziness. She has no history of similar symptoms. She sustained a myocardial infarction 2 months ago. She has hypertension and type 2 diabetes mellitus. Current medications include insulin and clonidine. She appears anxious. Her temperature is 37.1°C (98.8°F), pulse is 100/min, respirations are 18/min, and blood pressure is 160/90 mm Hg in both arms. A left carotid bruit is heard. Pulses are decreased in the lower extremities. Atherosclerotic involvement of which of the following arteries is the most likely cause of this patient's symptoms?

- ☐ A) Brachiocephalic
- ☐ B) Common carotid
- ☐ C) External carotid
- ☐ D) Internal carotid
- ☐ E) Middle cerebral
- ☐ F) Subclavian
- ☐ G) Vertebral



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

41. A 37-year-old woman with alcoholism is hospitalized for detoxification. On the second day after admission, she has an impaired sensorium and a decreased attention span during examination. She appears frightened and disoriented and shrieks, "Make the snakes go away." Her temperature is 38.3°C (101°F), pulse is 110/min, and blood pressure is 190/120 mm Hg. Which of the following is the most appropriate next step in management?

- ☐ A) CT scan of the head
- ☐ B) Oral chlordiazepoxide therapy
- ☐ C) Oral thioridazine therapy
- ☐ D) Intravenous antibiotic therapy
- ☐ E) Intravenous administration of vitamin B₁ (thiamine)



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

42. An otherwise healthy 52-year-old woman is brought to the emergency department 3 hours after the sudden onset of severe low back pain and severe shooting pain down her left buttock and leg. The pain began when she arose quickly from a seated position to open a sliding glass door. Two analgesic tablets leftover from a dental procedure provided no relief. She is an avid cyclist. On arrival, intravenous administration of morphine is begun and provides minimal relief. The patient notes numbness over the perineum when she attempts to void. She is reluctant to move from a flexed, seated position to a stretcher because of the pain. Muscle strength is 3/5 on great toe extension, dorsiflexion, and plantar flexion of the left foot. Knee jerk is present on the left. Achilles tendon reflex is absent on the left. An MRI of the lumbosacral spine shows a large central herniation at L4–5 that compresses the L5 and S1 nerves on the left. Which of the following is the most appropriate next step in management?

- ☐ A) Bone scan
- ☐ B) Physical therapy
- ☐ C) Bed rest and analgesic therapy
- ☐ D) Intravenous administration of muscle relaxants
- ☐ E) Lumbar laminectomy



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

43. A 32-year-old man comes to the physician 2 hours after the onset of dizziness, nausea, and unsteadiness. He had a whiplash neck injury 2 years ago and is still receiving monthly chiropractic manipulations. His voice is hoarse. There is left ptosis and nystagmus. The left pupil is 2 mm smaller than the right. There is decreased sensation to pain over the left side of the face and right extremities. He has ataxia on the left. Mental status examination shows no abnormalities. Which of the following arteries is most likely to be occluded?

- | | |
|--|---|
| ☐ A) Basilar | ☐ H) Right anterior cerebral |
| ☐ B) Left anterior cerebral | ☐ I) Right carotid |
| ☐ C) Left carotid | ☐ J) Right middle cerebral |
| ☐ D) Left middle cerebral | ☐ K) Right ophthalmic |
| ☐ E) Left ophthalmic | ☐ L) Right posterior cerebral |
| ☐ F) Left posterior cerebral | ☐ M) Right vertebral |
| ☐ G) Left vertebral | |



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

44. A 37-year-old woman comes to the physician because of a 4-month history of headaches and blurred vision. She has no history of serious illness and takes no medications. Menses occur at irregular 2- to 10-week intervals. She has had an 18-kg (40-lb) weight gain during the past 7 months. She is 163 cm (5 ft 4 in) tall and now weighs 86 kg (190 lb); BMI is 33 kg/m². Her temperature is 37°C (98.6°F). Visual acuity is 20/20 bilaterally. Visual field testing shows no abnormalities. Funduscopy examination shows blurred optic discs; there are no spontaneous venous pulsations. The remainder of the examination shows no abnormalities. A CT scan of the head shows small lateral ventricles. On lumbar puncture, the opening pressure is 290 mm H₂O; cerebrospinal fluid (CSF) analysis shows no leukocytes or erythrocytes. Which of the following is the most likely mechanism of these findings?

- ☐ A) Aqueductal stenosis
- ☐ B) Cerebellar astrocytoma
- ☐ C) Choroid plexus papilloma
- ☐ D) Impaired resorption of CSF
- ☐ E) Lateral venous sinus thrombosis



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

45. A 6-year-old boy is brought to the emergency department 20 minutes after a motor vehicle collision in which he was the unrestrained front seat passenger; he lost consciousness momentarily. Examination shows a 2 x 2-cm area of soft-tissue swelling over the forehead, a small abrasion on the left cheek, and an abrasion on the right elbow. Neurologic findings are normal. Which of the following is the most likely diagnosis?

- ☐ A) Cerebral concussion
- ☐ B) Cervical spine fracture
- ☐ C) Closed head injury
- ☐ D) Epidural hematoma
- ☐ E) Facial bone fracture
- ☐ F) Frontal skull fracture



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

46. A previously healthy 6-month-old girl is brought to the emergency department by ambulance because of difficulty breathing. Her mother says that she fell from the couch to a carpeted floor yesterday. She cried immediately and was fine until several hours later when she could not be aroused and seemed to be gasping for breath. Her mother noticed some emesis in the bed. On arrival, she is lethargic. Her temperature is 36.5°C(97.7°F), pulse is 130/min, respirations are 36/min, and blood pressure is 92/60 mm Hg. Examination shows a bulging anterior fontanelle. There are bilateral retinal hemorrhages. Neurologic examination shows no focal findings. A CT scan of the head shows blood along the falx. Which of the following is the most likely diagnosis?

- | | |
|---|--|
| ☐ A) Basilar skull fracture | ☐ H) Facial bone fracture |
| ☐ B) Caput succedaneum | ☐ I) Frontal skull fracture |
| ☐ C) Cephalhematoma | ☐ J) Intracerebral hematoma |
| ☐ D) Cervical spine fracture | ☐ K) Orbital blow-out fracture |
| ☐ E) Depressed skull fracture | ☐ L) Shaken baby syndrome |
| ☐ F) Diffuse axonal injury | ☐ M) Skull fracture over the middle meningeal artery |
| ☐ G) Epidural hematoma | ☐ N) Subdural hematoma |



47. A 67-year-old woman comes to the physician because of a 7-month history of progressive bilateral leg pain. She describes bilateral buttock, posterior thigh, and calf pain that begins after standing or walking for 10 minutes. It is relieved in 2 minutes by bending forward at the waist or sitting. She has coronary artery disease and underwent coronary artery bypass grafting 2 years ago. Neurologic examination shows no abnormalities. Which of the following is the most appropriate next step in diagnosis?
- ☐ A) Carotid duplex ultrasonography
 - ☐ B) Doppler flow studies of the lower extremities
 - ☐ C) Electromyography and nerve conduction studies
 - ☐ D) MRI of the lumbar spine
 - ☐ E) Myelography



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

48. A 37-year-old woman has had throbbing right-sided headaches that began gradually 6 weeks ago and have increased in frequency and severity. The headaches typically begin in the morning shortly after arising, last up to 1 hour, and occur several times daily. Acetaminophen has provided partial relief. She is irritable and has been unable to keep up daily activities. She appears upset. Neurologic examination shows a persistent attention deficit and left arm drift. Which of the following is the most likely diagnosis?

- | | |
|---|--|
| ☐ A) Brain tumor | ☐ G) Migraine |
| ☐ B) Cerebral hemorrhage | ☐ H) Occipital neuralgia |
| ☐ C) Cerebral nerve root impingement | ☐ I) Subarachnoid hemorrhage |
| ☐ D) Cluster headache | ☐ J) Subdural hematoma |
| ☐ E) Idiopathic intracranial hypertension | ☐ K) Temporal arteritis |
| ☐ F) Increased intracranial pressure | ☐ L) Tension-type headache |



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause

49. A 77-year-old woman comes to the physician because of a 3-year history of increasingly severe pain, swelling, and numbness of her left arm and shoulder; during the past 8 weeks, she has had difficulty moving her left arm because of these symptoms. Fifteen years ago, she underwent a left radical mastectomy followed by radiation therapy of the chest wall for breast cancer. There has been no evidence of recurrence of breast cancer. She has hypertension treated with atenolol. Her only other medication is ibuprofen as needed for pain. Her temperature is 37°C (98.6°F), pulse is 78/min, respirations are 16/min, and blood pressure is 110/70 mm Hg. Examination shows no neck, axillary, or chest wall masses. There is 2+ pitting edema of the entire left upper extremity. Examination of the right upper extremity shows no abnormalities. Radial and ulnar pulses are 2+ and equal bilaterally. Sensation to pinprick and motor function of the left upper extremity are absent. A photograph of the chest wall and left upper extremity is shown. An MRI of the left upper extremity and shoulder shows fibrosis but no masses. Which of the following is the most likely cause of this patient's symptoms?

- ☐ A) Entrapment of a cervical nerve root
- ☐ B) Radiation-induced brachial plexopathy
- ☐ C) Reflex sympathetic dystrophy
- ☐ D) Spinal stenosis
- ☐ E) Syringomyelia



50. An 82-year-old man is brought to the emergency department 1 hour after he collapsed while walking in his garden. He has hypercholesterolemia. He has smoked two packs of cigarettes daily for 60 years. His temperature is 37°C (98.6°F), pulse is 96/min and irregularly irregular, respirations are 18/min, and blood pressure is 150/90 mm Hg. He is awake but is unable to speak due to facial and tongue paralysis. He is able to blink his eyes and move his eyes vertically on command. He cannot move his eyes laterally. Examination shows complete paralysis of all extremities. Sensation to touch is intact. He undergoes endotracheal intubation for airway protection, and intravenous hydration is begun. An MRI of the brain shows extensive infarction of the base of the pons due to basilar artery occlusion. Conservative treatment is recommended. Although he cannot speak, he can communicate clearly by means of a code using eye blinks. The patient clearly communicates to his physicians repeatedly that he wants no life support or artificial hydration, and wants to die because of his poor prognosis for recovery. His wife and two sons agree with this decision but an estranged daughter does not and is threatening legal action. Which of the following is the most appropriate next step in management?

- ☐ A) A court order must be obtained to determine what life-sustaining measures, if any, may be withdrawn
- ☐ B) An ethics team consultation should be obtained and their decision followed concerning what life support measures, if any, may be withdrawn
- ☐ C) The patient must be maintained on artificial life support until he dies of natural causes, because there is no consensus in his immediate family
- ☐ D) The patient should be made do-not-resuscitate (DNR) and should be extubated, but all other life-sustaining measures, including artificial hydration, should be continued
- ☐ E) The patient should be made DNR and extubated, and fluids should be withdrawn as he wishes



Previous



Next

<https://t.me/USMLENBME2CK>

Lab Values



Calculator



Review



Help



Pause